

VIATRINIL 3mg/3mL Solution for Injection/ Infusion

Granisetron 3mg/3mL

Presentation:

VIATRINIL 3mg/3mL Solution for Injection/ Infusion is a sterile, clear, colourless, odourless, free from visible particles injection solution filled into amber glass ampoules.

Composition:

Each 3mL of injection solution contains of active ingredient, 3mg granisetron present as the hydrochloride. Other excipients include Sodium Chloride, Citric Acid Monohydrate, Hydrochloric Acid or Sodium Hydroxide (pH adjustment) and Water for Injection. The drug product is preservative free.

Pharmacodynamic:

Granisetron is a potent and highly selective 5-hydroxytryptamin (5-HT₃) receptor antagonist. Binding studies have shown that granisetron has negligible affinity for other receptors, including 5-HT and dopamine D₂ binding sites.

Granisetron effectively prevents nausea and vomiting caused by chemotherapy or radiotherapy.

Pharmacokinetics:

General characteristics:

Distribution: Granisetron is distributed in the main part of the body with a mean volume of distribution of 3L/kg; plasma protein binding is approximately 65%.

Biotransformation: Biotransformation pathways involve N-demethylation and aromatic ring oxidation followed by conjugation.

Elimination: Clearance is predominantly by hepatic metabolism. Urinary excretion of unchanged granisetron averages 12% of dose whilst that of metabolites amounts to about 47% of dose. The remainder is excreted in faeces as metabolites. Mean plasma half-life in patients is approximately 9 hours, with a wide inter-subject variability.

Characteristics in patients:

The plasma concentration of granisetron is not clearly correlated with anti-emetic efficacy. Clinical benefit may be conferred even when granisetron is not detectable in plasma.

In elderly subjects after single intravenous doses, pharmacokinetic parameters were within the range found for non-elderly subjects. In patients with severe

renal failure, data indicate that pharmacokinetic parameters after a single intravenous dose are generally similar to those in normal subjects. In patients with hepatic impairment due to neoplastic liver involvement, total plasma clearance of an intravenous dose was approximately halved compared to patients without hepatic involvement. Despite these changes, no dosage adjustment is necessary.

Following intravenous single dose in children, pharmacokinetic parameters (volume of distribution and plasma clearance) are equal to those in adults, according to body weight.

Indications:

VIATRINIL is indicated for the prevention and treatment (control) of
a) acute and delayed nausea and vomiting associated with chemotherapy
and radiotherapy

b) post-operative nausea and vomiting.

Dosage and Administration:

Chemotherapy Induced Nausea and Vomiting (CINV):

Adults:

Prevention: A dose of 1-3 mg (10-40mcg/kg) of **VIATRINIL** should be administered either as a slow intravenous injection (over 30 seconds) or as an intravenous infusion diluted in 20 to 50 mL infusion fluid and administered over 5 minutes, prior to the start of chemotherapy.

Treatment: A dose of 1-3 mg (10-40mcg/kg) **VIATRINIL** should be administered either as a slow intravenous injection (over 30 seconds) or as an intravenous infusion diluted in 20 to 50 mL infusion fluid and administered over 5 minutes. Further treatment doses of **VIATRINIL** may be administered, if required, at least 10 minutes apart. The maximum dose of **VIATRINIL** to be administered over 24 hours should not exceed 9 mg.

Paediatrics:

Prevention and treatment: A dose of 10-40 mcg/kg body weight (up to 3 mg) should be administered as an intravenous infusion, diluted in 10 to 30 ml infusion fluid and administered over 5 minutes prior to the start of chemotherapy. One additional dose may be administered within a 24-hour period if required. This additional dose should not be administered until at least 10 minutes after the initial infusion.

Radiotherapy Induced Nausea and Vomiting (RINV):

Adults:

Prevention: A dose of 1-3 mg (10-40mcg/kg) of **VIATRINIL** should be administered either as a slow intravenous injection (over 30 seconds) or as an intravenous infusion diluted in 20 to 50 mL infusion fluid and administered over 5 minutes, prior to the start of radiotherapy.

Treatment: There is insufficient information to recommend the intravenous administration of **VIATRINIL** in the treatment of RINV in adult patients.

Paediatrics:

There is insufficient information to recommend intravenous administration of **VIATRINIL** in the prevention and treatment of RINV in children.

Post-operative Nausea and Vomiting (PONV):

Adults:

Prevention: A dose of 1 mg (10mcg/kg) of **VIATRINIL** should be administered as a slow intravenous injection (over 30 seconds) prior to induction of anesthesia.

Treatment: A dose of 1 mg (10mcg/kg) of **VIATRINIL** should be administered by slow intravenous injection (over 30 seconds). The maximum dose for patients undergoing anaesthesia for surgery is a total dose of 3 mg of **VIATRINIL** injection in one day.

Paediatrics:

There is insufficient information to recommend intravenous administration of **VIATRINIL** in the prevention and treatment of post-operative nausea and vomiting in paediatric patients.

Dosage for Special Populations:

Elderly:

No dosage adjustment is necessary for elderly patients.

Patients with renal impairment:

No dosage adjustment is necessary for patients with renal impairment.

Patients with hepatic impairment:

No dosage adjustment is necessary for patients with hepatic impairment.

Contraindications:

Hypersensitivity to granisetron or to any of the excipients.

Special Warnings and Precautions:

As granisetron may reduce the motility of intestines, patients with signs of sub-acute intestinal obstruction should be monitored carefully following administration of granisetron injection.

No special precautions are required for the elderly or renally or hepatically impaired patient.

Effects on ability to drive and use machines:

There has been no evidence from human studies that granisetron has any adverse effect on alertness.

This medicinal product contains 0.154 mmol (3.540mg) sodium per mL. If the patient is on a low sodium diet this must be taken into account, if the dose exceeds 6.5mL corresponding to 1 mmol sodium (22mg sodium).

Use in pregnancy & lactation:

Whilst animal studies have shown no teratogenic effects, there is no experience of granisetron in human pregnancy. Therefore, granisetron injection should not be administered to women who are pregnant unless there are compelling clinical reasons.

There are no data on the excretion of granisetron in breast milk. Breast feeding should therefore be discontinued during therapy.

Drug Interactions:

Specific studies demonstrated that granisetron does not interact with benzodiazepines, neuroleptics and medicinal products for gastric ulcers, which are normally prescribed together with medicinal products for nausea and vomiting.

No interaction studies have been performed between granisetron and anaesthetics. There are no reports on interaction between the commonly administered anaesthetics and analgesics with granisetron.

Incompatibilities:

VIATRINIL should not be mixed with other medicinal product except those mentioned in section **Preparing Solution for Administration**.

Prophylactic administration of **VIATRINIL** should be completed prior to the start of chemotherapy.

Adverse Reactions:

Granisetron has been generally well tolerated in human studies. As reported with other drugs of this class, headache and constipation have been the most

frequently noted adverse events but the majority have been mild or moderate in nature.

Rare cases of hypersensitivity reaction, occasionally severe (e.g. anaphylaxis) have been reported. Other allergic reactions including minor skin rashes have also been reported.

In clinical trials, transient increases in hepatic transaminases, generally within the normal range, have been seen.

Dystonias and dyskinesias have been reported with medicines in the 5-HT₃ antagonist class. Such events have been reported rarely with granisetron.

Overdosage:

Overdosage up to 30mg of granisetron intravenously has been reported with symptoms of slight headache but no other sequelae were observed.

There is no specific antidote for granisetron. In the case of overdosage, symptomatic treatment should be given.

Preparing Solution for Administration:

Adults:

To prepare a dose of 3mg, 3mL of **VIATRINIL** is withdrawn from the ampoule and diluted either to 15ml with 0.9% w/v Sodium Chloride Injection (for bolus administration) or in infusion fluid to a total volume of 20 to 50mL in any of the following solutions:

0.9% w/v Sodium Chloride Solution for Infusion;

0.18% w/v Sodium Chloride and 4% w/v Glucose Solution for Infusion;

5% w/v Glucose Solution for Infusion;

Hartmann's Solution for Infusion;

1.85% w/v Sodium Lactate Solution for Infusion; or

10% Mannitol Solution for Infusion.

No other diluents should be used.

VIATRINIL can be admixed with Dexamethasone Sodium Phosphate 4mg/mL Injection at concentration of 10 - 60µg/mL granisetron and 80 - 480µg/mL dexamethasone phosphate injection in either 0.9% w/v Sodium Chloride Solution for Infusion or 5% w/v Glucose intravenous Solution for Infusion.

To prepare a dose of 1mg, 1mL should be withdrawn from the ampoule and diluted to 5ml with 0.9% w/v Sodium Chloride Injection. No other diluent should be used.

Paediatrics:

To prepare the dose of 40µg/kg the appropriate volume (up to 3mL) is withdrawn from the ampoule and diluted with infusion fluid (as for adults) to a total volume of 10 to 30mL. Proportion of infusion fluid to granisetron should be at least 5mL solution to 1mg granisetron.

Ideally, intravenous infusions of **VIATRINIL** should be prepared at the time of administration.

After dilution, the shelf life is 24 hours when stored at 15-25°C in normal indoor illumination protected from direct sunlight. It must not be used after 24 hours.

If to be stored after preparation, **VIATRINIL** infusions must be prepared under appropriate aseptic conditions. Discard any unused portion after dilution.

Storage Conditions:

Store below 30°C. Store in the original package in order to protect from light. Ideally, intravenous infusions of **VIATRINIL** should be prepared at the time of administration.

After dilution/ reconstitution, the shelf life is 24 hours when stored at 15-25°C in normal indoor illumination protected from direct sunlight. It must not be used after 24 hours. If to be stored after preparation, granisetron infusions must be prepared under appropriate aseptic conditions. Discard any unused portion after dilution.

How Supplied:

VIATRINIL 3mg/3mL Solution for Injection/ Infusion is supplied in 3mL amber glass ampoules. 5 ampoules are held by PVC tray and packaged with a product insert into a carton box.

Date of Revision of Package Insert: June 2023.

Manufacturer:

VIANEX S.A.,
PLANT A,
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14451 Metamorphossi, Attiki, Greece.